

HOW TO OPEN AN ADULT CARE FACILITY or ASSISTED LIVING RESIDENCE in New York

The license stack — adult home / enriched housing, ALR certification & the ALP.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate & fee should be re-verified before you rely on it.

The New York model — read this first

New York does NOT have a single “assisted living license.” It has a stack of them, and understanding the stack is the first and most important step. The base license is an adult care facility (ACF): a residence providing room, board, housekeeping, supervision, and — in most types — personal care to five or more adults who cannot live fully independently but don't need the continuous medical or nursing care of a hospital or nursing home. On top of the ACF, an operator may add certification as an assisted living residence (ALR) — the modern model that adds 24-hour on-site monitoring, an individualized service plan, and daily food service. And an operator may add the Assisted Living Program (ALP), a Medicaid service overlay. A single building can hold one license, two layered together, or all three — and which combination you hold determines who can live there and who pays.

***The license stack is the whole game.** ACF, ALR, and ALP are different things with different payers. Decide which you need BEFORE you design the building or the budget — misreading the stack is the most common and most expensive planning mistake in New York. This guide focuses on the adult home / enriched housing base plus ALR certification (the core “assisted living” path) and is honest about how you actually get paid.*

***The two hard New York realities.** (1) Timeline: a new adult care facility can take 18 months to two years to move through construction, local permits, and DOH licensing — plan your cash flow for it. (2) The ALP is capped: Medicaid-funded services flow only through the Assisted Living Program, which is capped at 4,200 residents statewide and released only through periodic competitive solicitations — you can't simply apply whenever you want.*

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It's written for someone who has never opened a facility — every time we name a thing, we stop, break it down, and tell you exactly where to go and what to ask. It is general information, not legal, tax, or clinical advice; confirm every step, figure, and contact with DOH and your Local Department of Social Services (see Part 12) before you rely on it.

The license stack — what each one means

Layer	What it is	Who pays
Adult Care Facility (ACF)	Base license — adult home (Part 487) or enriched housing (Part 488). Room, board, supervision, personal care.	Private pay, or SSI + State Supplement for room & board
Assisted Living Residence (ALR)	Certification on top of an adult home/EHP (PHL 46-B; 10 NYCRR 1001), with Enhanced (EALR) and Special Needs (SNALR) sub-types.	ALR-only is essentially private pay
Assisted Living Program (ALP)	Medicaid service overlay (Part 494); capped statewide and released through periodic competitive solicitations.	Medicaid for services + SSI + State Supplement for room & board
Family-Type Home (FTHA)	Up to 4 adults; licensed at the Local DSS level (Part 489). The small-scale entry point.	Private pay or SSI + State Supplement

The journey at a glance

Ten milestones take you from idea to an operating, certificated facility.

1	Decide your model & set up the business	Which base license; whether to add ALR/ALP; entity, EIN, NPI, insurance (Parts 3–4)
2	Have a pre-application conference with DOH	New York recommends it before you apply — take it (Part 5)
3	Secure a location & confirm zoning	Think ahead to DOH architectural review (Part 6)
4	Submit the Common Application	Judged on public need, financial feasibility, and character & competence (Part 5)
5	Complete architectural / plan review	Plus local building, fire & sanitation approvals (Part 6)
6	Get Part I, then Part II	Establishment, then program information within the required window (Part 7)
7	Add ALR certification (if desired)	Layered on your adult home / enriched housing base (Part 8)
8	Pursue the ALP (if desired)	Respond to a DOH RFA when capped beds are released (Part 8)
9	Hire, screen & get survey-ready	Staff, policies, records — before you admit (Parts 9–10)
10	Get your operating certificate & open	Both Part I and Part II approved before admitting; connect residents to SSL/SSP & ALP (Parts 7, 11)

Rule of thumb. Take the pre-application conference — it can save months in a long process. Decide your full license stack before you design the building. And never admit a resident until BOTH Part I and Part II are approved and your operating certificate is issued.

PART 1

What This Business Is

In short: *A licensed residence providing room, board, supervision, and personal care to adults who can't live fully independently but don't need a nursing home — with optional ALR certification for the modern assisted-living model and the ALP Medicaid overlay. Paid by SSI + State Supplement for room and board, and Medicaid (ALP) or private pay for services.*

An adult care facility gives older adults and people with disabilities a place to live where room, board, housekeeping, supervision, and personal care are provided — but not the continuous nursing or medical care of a hospital or nursing home. The two most common base licenses are the adult home (congregate rooms, personal care, at least five residents) and the enriched housing program (apartment-style units, community-integrated, at least one hot congregate meal a day). Enriched housing is often the substrate operators pair with the ALP because its apartment configuration fits home-and-community-based-services rules.

Be clear-eyed about what kind of business this is. Like an assisted living facility anywhere, it's a real-estate and operations business first — a building, around-the-clock supervision, meals, and the safety of people who live under your roof — and in New York it carries an unusually long approval timeline. That makes it capital-intensive and slow to open, but it builds a durable asset and a steady, occupancy-based revenue stream once you're established.

The layers you can add

- **Assisted Living Residence (ALR) certification:** the modern 2004 model layered on an adult home / enriched housing base — 24-hour on-site monitoring, an individualized service plan, and daily food service, with Enhanced (EALR, aging-in-place) and Special Needs (SNALR, dementia) sub-types.
- **The Assisted Living Program (ALP):** a Medicaid service overlay — but capped at 4,200 residents statewide and released only through periodic competitive Requests for Applications.
- **Family-Type Home for Adults (FTHA):** up to four adults, licensed by your Local DSS — the simpler small-scale entry point if you're starting tiny.

Key terms, in plain English. ACF = Adult Care Facility (your base license). Adult home / EHP = the two common base types (congregate rooms / apartments). ALR = Assisted Living Residence (a certification you add). EALR / SNALR = the Enhanced (aging-in-place) and Special Needs (dementia) ALR sub-types. ALP = Assisted Living Program (the capped Medicaid service overlay). DOH = NY Dept. of Health (licenses you). Local DSS = your Local Department of Social Services (Medicaid, ALP approval, SSI/SSP). SSP = the NY State Supplement Program (room & board). UAS-NY = the Uniform Assessment System. Justice Center = the oversight body for reportable incidents. NPI = National Provider Identifier.

Why this business — and why now

New York's older population is large and growing, and families overwhelmingly want a residential, home-like setting short of a nursing home. The state's SSI + State Supplement structure funds room and board for low-income residents, and the ALP funds services for those who qualify — so a well-run, well-located facility has real, ongoing demand and defined public funding behind it. The trade-off is New York's genuinely heavy front end: a long approval, a layered license, and close scrutiny of your finances and character. Operators who plan for that and capitalize accordingly build a durable business.

Be honest about the timeline and the ALP cap. Two things sink underprepared New York operators: assuming a facility opens fast (it can take 18–24 months), and assuming Medicaid (the ALP) is available on demand (it's capped and competitive). Plan cash flow for a long, revenue-free approval period, and treat ALP capacity as something you watch and compete for — not something you assume.

PART 2

Who Regulates You

In short: *The DOH Bureau of Licensure and Certification licenses and surveys you, working with your Local Department of Social Services on Medicaid, the ALP, and SSI/State-Supplement matters. The Justice Center and the Long-Term Care Ombudsman add oversight.*

NY DOH — Bureau of Licensure and Certification (your licensor)

The Department of Health licenses adult care facilities through the Adult Care Facility Common Application (working with the appropriate DOH regional office), conducts the architectural review, issues your operating certificate, and surveys you after you open. This is your primary regulator under SSL Article 7, 18 NYCRR Parts 485–490, and (for the ALR) PHL Article 46-B and 10 NYCRR Part 1001.

Your Local Department of Social Services (DSS)

The Local DSS handles Medicaid, ALP approval, and SSI/State-Supplement matters, and is aware of people needing placement. For the ALP and for your residents' room-and-board funding, the Local DSS is a central relationship.

The Justice Center & the Long-Term Care Ombudsman

If your facility is subject to the Justice Center for the Protection of People with Special Needs, you check prospective staff against the applicable exclusion systems and report reportable incidents to the Vulnerable Persons' Central Register. The Long-Term Care Ombudsman advocates for residents — post the Ombudsman's contact information with your grievance procedure.

What this means for you: New York judges the OPERATOR, not just the building. Your finances, your ownership, and your character and competence are examined closely (Part 5), and out-of-state health-facility affiliations trigger an out-of-state compliance review. Assemble your ownership and financial disclosures carefully — incomplete or weak submissions are the most common cause of delay or denial.

PART 3

Form Your Business & Insure — Step by Step

In short: *This is where most first-timers get stuck. We break each piece all the way down — what it is, where to go, exactly what to do, and what to have in hand.*

3.1 Form your legal entity (an LLC)

An “entity” is the legal business — almost always a Limited Liability Company (LLC) — that holds your license or registration, your contracts, and your bank account, and that shields your personal assets if the business is ever sued. In New York you form the LLC through the Department of State, Division of Corporations. DOH reviews the legal structure closely, and if you pursue the ALP, the ownership of both the facility and the home-care service component is examined — so keep your entity information consistent everywhere. You'll name a “registered agent” (a person or service with a physical in-state address who receives legal mail — this can be you) and adopt an Operating Agreement (the internal document that sets out who owns what and who can make decisions).

- 1. File your Articles of Organization with the NY Department of State.** Online at dos.ny.gov — the LLC filing fee is \$200. Name the LLC and designate the Secretary of State as agent for service.
- 2. Complete the NY LLC newspaper publication.** New York requires a new LLC to publish a formation notice in two county-designated newspapers for six weeks and file a Certificate of Publication — budget for it (see the callout).
- 3. Adopt an Operating Agreement.** New York law requires LLCs to adopt a written Operating Agreement; your bank and DOH will want it.
- 4. File the biennial statement.** NY LLCs file a \$9 biennial statement with the Department of State every two years to stay current.

Budget for the New York LLC publication cost. *New York is one of the few states that makes a new LLC publish a formation notice in two county-designated newspapers for six weeks — which can cost several hundred dollars upstate and well over a thousand (sometimes several thousand) in Manhattan. It's a real, often-surprising startup cost.*

3.2 Get your EIN — the free federal tax ID

An EIN (Employer Identification Number) is your business's federal tax number — think of it as a Social Security number for the company. It's free, takes about ten minutes, and you need it before you can open a bank account, run payroll, or enroll with any payer. Here's exactly how to get it:

- 1. Go to IRS.gov and open the EIN Assistant.** Search “Apply for an EIN online.” Apply directly on the IRS website — never pay a third-party site that charges for this free service.
- 2. Choose your entity and enter the responsible party.** Select “Limited Liability Company,” then give the responsible party's legal name and SSN or ITIN (usually you, the owner), plus your business name and address.
- 3. Finish in one sitting.** The online session times out after about 15 minutes of inactivity and can't be saved partway — have your details ready before you start.
- 4. Save your EIN and the CP-575 letter.** You get the EIN on the spot and can download the official CP-575 confirmation letter — save both; your bank and every payer will ask for them.

3.3 Get your NPI — the national provider ID

An NPI (National Provider Identifier) is a free, ten-digit ID number that health-care payers use to identify your organization. You'll use it if you pursue the Assisted Living Program (ALP), the Medicaid service overlay; an ALR-only facility that is essentially private pay may not need it. Your AGENCY needs a Type 2 (Organizational) NPI — which is different from the Type 1 (individual) NPI a single person would get.

- 1. Create an I&A account.** At nppes.cms.hhs.gov, set up an Identity & Access (I&A) account for whoever will manage the number — usually an owner.

2. Start a Type 2 (Organizational) application. Log into NPPES and choose Type 2, not Type 1. Type 1 is for an individual person; your business is a Type 2 organization.

3. Enter your legal name, EIN, and address, and pick your taxonomy. Choose the assisted-living / residential-care taxonomy that fits your facility — you'll need this if you pursue the ALP (the Medicaid overlay). The “taxonomy” is the code that says what kind of provider you are — confirm the exact one your Medicaid program wants before you submit.

4. Submit — it's free and quick. It usually processes in about 10 business days. Save the NPI; you'll use it on every enrollment and claim.

3.4 Open a business bank account

Keep the business's money completely separate from your personal money — it protects your liability shield and makes payroll, taxes, and payer deposits clean. Open the account once your entity and EIN are set up.

What to bring to the bank:

- Your EIN confirmation (CP-575) from the IRS
- Your filed Articles of Organization / Certificate of Formation and the entity ID number
- Your signed Operating Agreement
- A Beneficial Ownership form (the bank collects details on anyone owning 25%+ plus a control person)
- A government photo ID, SSN, date of birth, and home address for each owner/signer
- An opening deposit (often around \$100)

Where to go: a bank with lots of New York branches — Chase, Citibank, Bank of America, M&T, KeyBank, or a local New York credit union. Home care is payroll-heavy — you pay caregivers weekly, but Medicaid and insurers pay you weeks later — so ask specifically about a business line of credit or invoice financing to bridge payroll before you scale.

3.5 Get your insurance — what you need, who sells it, and what to ask

An adult care facility houses and supervises vulnerable residents around the clock, so you need coverage built for a licensed residential setting — and New York requires proof of workers' compensation AND disability coverage:

Coverage	What it protects against	Must-have?
General & Professional Liability	Injury or property damage, plus claims about the CARE and supervision	Yes — foundational
Abuse & Molestation	Allegations of abuse of a resident — general liability EXCLUDES this	Yes — high exposure
Property & Business Interruption	Fire and damage to the building, plus lost income	Yes
Directors & Officers / Management	Regulatory, employment, and governance claims	Recommended
Workers' Compensation & Disability	Staff injuries; New York mandates disability coverage too	Required in NY for employees
Non-Owned & Hired Auto	Staff driving residents or on facility errands	Recommended

New York requires disability coverage too. On top of workers' comp, New York employers must carry statutory disability (and Paid Family Leave) coverage — DOH will want proof. Your payroll provider or broker sets it up; factor it into your labor cost.

Real places to get it

Buy through an independent agent or broker who writes SENIOR-LIVING / adult-care programs — this is a specialty niche. Carriers and programs active in senior living include Philadelphia Insurance (PHLY), CNA, The Hartford, Great American, Markel, and senior-living specialty programs. Find a local NY agent at [trustedchoice.com](https://www.trustedchoice.com) and ask specifically for an adult care facility / assisted living program. Confirm the carrier writes NY adult care facilities before you rely on a quote.

What to ask the insurance agent

“Do you write NY adult care facilities / assisted living residences — which carriers, and what's the minimum premium?”

“Is abuse & molestation included, at what limit, and is it a sub-limit or a full limit?”

“Does professional liability reflect our personal-care and supervision model?”

“What limits do DOH, my lender, and (if applicable) the ALP expect?”

“Can you set up New York workers' comp AND statutory disability coverage?”

3.6 Workers' comp, disability & keeping your structure consistent

Two New York-specific business items catch new operators. First, coverage: New York requires proof of BOTH workers' compensation and statutory disability (with Paid Family Leave) coverage before you can be licensed — set these up with your payroll provider or broker early, because DOH will ask for the certificates. Second, consistency: DOH reviews your legal structure closely, and if you pursue the ALP, the ownership of BOTH the facility and the home-care service component is examined. Keep your entity name, ownership percentages, and addresses identical across your Articles of Organization, your EIN letter, your insurance, and your Common Application — a mismatch between documents is a common, avoidable source of delay in a review that already scrutinizes character and competence closely. It's worth having your accountant and attorney confirm the structure once, up front, so the same clean picture appears everywhere DOH looks.

PART 4

Choose Your Model & the License Stack

In short: *Decide your base license (adult home or enriched housing), whether to add ALR certification, and whether to pursue the ALP — because the combination determines your building, your residents, and who pays. Decide it all before you design the building.*

- **Adult home (Part 487):** congregate rooms, personal care, at least five residents — the classic base license.
- **Enriched housing program (Part 488):** apartment-style units, community-integrated, at least one hot congregate meal a day — often paired with the ALP because its apartment configuration fits the home-and-community-based-services rules.
- **Add ALR certification?** for the modern assisted-living model (24-hour monitoring, service plans, daily food service), with EALR (aging-in-place) and SNALR (dementia) sub-types.
- **Pursue the ALP?** only if Medicaid-funded services are central to your plan — and only when DOH releases capped beds through a competitive solicitation.

Starting small? The family-type home. If you're starting tiny, the family-type home for adults (up to four adults, licensed by your Local DSS under Part 489) is a simpler entry point than the full ACF process. Many operators cut their teeth there.

What this means for you: the model choice is really a capital decision. An adult home built with congregate rooms is generally less to build than enriched-housing apartments, but enriched housing is what pairs with the ALP for Medicaid-funded services — so if the ALP is central to your plan, the pricier apartment configuration may be the right long-term investment. And every layer you add (ALR, then EALR or SNALR, then the ALP) adds building, staffing, training, and disclosure requirements on top of the base. The operators who do best decide the FULL stack they're aiming for on day one and design the building and budget for the end state, rather than licensing a base home and discovering later that adding a layer means rebuilding or re-surveying. Map your intended stack before you draw a single floor plan.

PART 5

The DOH Application — Need, Feasibility, Character

In short: Adult care facilities are licensed through the Common Application, judged on three things: public need, financial feasibility, and character and competence. Take the recommended pre-application conference — it can save months.

New York recommends a pre-application conference with DOH before you apply — take it; it can save months in a long process. Then the Common Application is judged on three tests:

- 1. Public need.** DOH's assessment that the facility is needed in your area — part of why location and market matter.
- 2. Financial feasibility.** Showing you can fund the project and operate it — typically with a certified public accountant confirming your resources and estimating operating costs for the first and third years.
- 3. Character and competence.** A detailed questionnaire completed by ALL owners covering education, employment, licenses, health-facility affiliations in and outside New York, and any legal actions. Out-of-state affiliations trigger an out-of-state compliance review.

Weak submissions are the #1 cause of delay. Incomplete owner disclosures or unverified finances are the most common reasons applications stall or are denied. Assemble the character-and-competence questionnaires and the CPA feasibility package carefully and completely before you file — and use the pre-application conference to confirm what a complete package looks like.

PART 6

Architectural Review, Building, Fire & Sanitation

In short: *If your project involves construction or renovation, DOH conducts an architectural / plan review against the standards for adult care facilities, and you obtain local building, fire, and sanitation approvals. Plan review is where long timelines get longer.*

DOH reviews your plans against the environmental and structural standards for adult care facilities, and you must attest to building to the New York State building codes. You may request approval to commence early construction at your own risk, but final approval is required before residents move in. In parallel, you obtain the local building, fire-safety, and sanitation approvals your municipality requires.

Design to the standards from the start. *The building's exits, fire-safety systems, unit configuration, and accessibility affect your capacity and the residents you can serve, and enriched housing has its own capacity rules tied to the building. Involve DOH and your architect early — plan review is where long timelines get longer, and a building that looks ready can still need changes to meet code for a licensed residence.*

What plan review looks at

- **Life safety and egress:** exits, fire-rated construction, sprinklers/alarms, and evacuation routes sized for residents who may need assistance to leave.
- **Unit and capacity configuration:** bedroom or apartment sizes, bathrooms, and the resident count the building can safely support — which sets your licensed capacity.
- **Common, dining and program space:** adequate dining, activity, and common areas for the residents you'll serve.
- **Accessibility:** accessible routes, bathrooms, and features throughout, per code.
- **Kitchen, sanitation and systems:** food-service, heating/cooling, water, and sanitation that pass both DOH and local health review.

Because these building elements set your capacity and your eligible residents, resolve them in the drawings — not after you've built. Ask DOH and your architect to confirm your intended capacity is achievable in your building before you commit, since discovering a capacity limit after construction is one of the most expensive surprises in the whole process.

PART 7

Part I, Part II & the Operating Certificate

In short: *The application runs in two parts: Part I is establishment (approving you and the project), Part II is program information (how it will operate). BOTH must be approved before you admit any residents, and DOH issues your operating certificate when the project is approved.*

- 1. Get Part I — establishment.** DOH's decision that you (the operator) and the project should be approved, based on need, feasibility, and character and competence.
- 2. Submit Part II — program information (Schedule 6).** Within the required window after Part I approval, describing how the facility will actually operate.
- 3. Both approved before you admit anyone.** You may not admit a single resident until BOTH Part I and Part II are approved and your operating certificate is issued.
- 4. Operate within your certificate.** It states your facility type, capacity, and certifications — post it, operate within it, and seek DOH approval before any change in operator, capacity, category, location, or certification that requires it. Renew it periodically.

What Part II (Schedule 6) actually describes is how the facility will run day to day: your admission and retention criteria, your staffing plan and organizational chart, your resident-services and case-management approach, your food service, your policies and resident-rights framework, and your emergency and disaster planning. In other words, Part I asks “should this operator and project exist?” and Part II asks “exactly how will it operate safely?” — so the same policy manual, staffing plan, and service systems you'll be surveyed on later are what you describe here. Building those systems well for Part II means you're most of the way to being survey-ready (Part 10), which is why experienced operators treat Part II as the operational blueprint of the business, not a form to rush.

Do not admit anyone until both parts are approved. *This is the single most serious sequencing rule: both Part I (establishment) and Part II (program) must be approved and your operating certificate issued before a single resident moves in. Admitting a resident on an incomplete approval is operating without a license.*

PART 8

Adding ALR Certification & the ALP

In short: To certify as an ALR you must first hold (or obtain at the same time) an adult home / enriched housing base. The ALP is different — a capped Medicaid program released only through periodic competitive solicitations.

8.1 ALR certification (layered on your base)

To certify as an assisted living residence, you must first hold — or obtain at the same time — an adult home or enriched housing operating certificate; the ALR is a certification layered on that base, with a character-and-competence review and the consumer-information and residency-agreement requirements of 10 NYCRR Part 1001. From there you can seek Enhanced (EALR) certification to admit and retain residents who need more physical assistance so they can age in place, or Special Needs (SNALR) certification for dementia care, each with additional program, staffing, training, and environmental requirements.

Build memory care in from the start. If memory care is part of your plan, decide early whether to certify as a Special Needs ALR (SNALR). New York treats dementia care as a distinct certification with additional staffing, training, environment (for example, secure settings for residents at risk of wandering), and disclosure requirements — build it in rather than retrofitting it.

8.2 The Assisted Living Program (ALP) — capped and competitive

The ALP is a Medicaid program with a statewide cap of 4,200 residents, and new ALP beds are released only through periodic competitive Requests for Applications announced by DOH. You cannot simply apply for ALP capacity whenever you wish — you respond to a solicitation using the Common Application. If Medicaid-funded services are central to your plan, watch DOH's announcements and be ready to apply when beds are offered.

PART 9

Staffing, Screening & Training

In short: Staff the facility for an organized 24-hour program of supervision and services, screen everyone before they serve residents, and train to New York's requirements — including the Justice Center and ALR/dementia training where they apply.

9.1 The staffing rules

- **Staff for a 24-hour program:** enough qualified people for your census and residents' assessed needs — an ALR provides 24-hour on-site monitoring.
- **Screen before they serve residents:** the criminal-history and abuse/registry checks required for adult care facility staff, plus staff health and tuberculosis screening.
- **Remember the medication distinction:** adult home staff ASSIST residents with the self-administration of medications rather than administering them as a nurse would (unless authorized under the facility's certification) — this is a key New York difference from a nursing home.
- **Justice Center obligations, where they apply:** check prospective staff against the applicable exclusion systems and train staff to report reportable incidents to the Vulnerable Persons' Central Register.
- **Train and document:** resident rights, incident recognition and reporting, infection control, emergency procedures, plus the ALR and dementia-specific training the rules require — document every screening, training, and competency.

9.2 Recruiting and keeping good caregivers — the real make-or-break

Every experienced agency owner will tell you the same thing: your business rises or falls on caregivers, not clients. Referral sources will send you all the clients you can handle — but only if you can reliably staff them. So treat recruiting and retention as your core operation, not an afterthought.

- **Recruit constantly:** post continuously (Indeed, local Facebook groups, word of mouth, and a caregiver-referral bonus), and make applying fast — the best caregivers take the first agency that calls them back and schedules an interview.
- **Hire for reliability and warmth:** specific skills can be trained; showing up on time and being kind cannot. Check references for dependability, not just experience.
- **Onboard properly:** a real orientation, a skills check-off before a caregiver works alone, and a written scope of what they may and may not do — this protects clients and builds the caregiver's confidence.
- **Retain deliberately:** consistent hours, quick and correct pay, being reachable when a caregiver has a problem, genuine recognition, and a small raise ladder. Turnover is the biggest hidden cost in home care — every caregiver who quits costs you recruiting, training, and often the client they served.

PART 10

Policies, Records & Getting Survey-Ready

In short: *You operate from written policies and procedures, and you set up resident records before your first admission. Being survey-ready on day one is the difference between a clean first inspection and an early plan of correction.*

Operate from written policies and procedures covering resident care and facility operation, mapped to 18 NYCRR Parts 485–490 and (for the ALR) 10 NYCRR Part 1001 — kept current and available to DOH. Set up resident records before your first admission: the admission or residency agreement, the assessment and individualized service plan, medication information and records of the assistance provided, case-management and progress notes, incident records, and the signed statement of resident rights (and, for an ALR, the Resident's Bill of Rights). Post the resident rights, the grievance procedure with Ombudsman contact information, and your emergency procedures.

10.1 Documentation & staying inspection-ready

Whatever your state requires, the agencies that pass surveys and payer audits cleanly all do the same thing: they keep complete, current files from day one instead of scrambling the week before an inspection. Build these habits now, because reconstructing records after the fact is nearly impossible:

- **A file for every caregiver:** the application, background checks, training and competency records, health/TB screening, and a signed acknowledgment of your policies — all dated and kept current.
- **A file for every client:** the assessment, the service/care plan, the signed service agreement, consents, ongoing visit notes, and any payer authorizations.
- **A visit record for every shift:** who worked, when they arrived and left, and what care was provided — and, for Medicaid, the matching EVV record.
- **An incident and complaint log:** every incident, injury, and complaint and exactly how you resolved it — inspectors look for a real, used quality-improvement process, not a binder that's never been opened.

PART 11

How You Get Paid — SSI, the State Supplement & the ALP

In short: New York separates the two halves of the bill. Room and board is paid from the resident's income — SSI plus the New York State Supplement for low-income residents. Care and services, for those who qualify, are paid by Medicaid — but only through the capped ALP, and Medicaid never pays room and board.

Room and board is paid from the resident's own income; for low-income residents that means SSI plus the New York State Supplement (SSP), administered by the Office of Temporary and Disability Assistance and set by “congregate care level.” Care and services, for those who qualify, are paid by Medicaid — but only through the Assisted Living Program, and Medicaid NEVER pays room and board. An ALR without the ALP is essentially private pay.

The ALP catch is real. The ALP is capped at 4,200 residents statewide, is not an entitlement, and forms a waitlist when full. An applicant must be assessed (through UAS-NY) as needing nursing-home level of care but not continuous nursing, and must be able to evacuate with assistance. Plan your payer mix around this — don't assume ALP Medicaid is available on demand.

11.1 Funding at a glance (2026 — verify current amounts)

Source	What it pays for	Amount / conditions
SSI + NY State Supplement (Congregate Level 3)	Room & board (enhanced residential / assisted living)	Up to ~\$1,688/month (individual, 2026); resident contributes income minus a \$262/month personal needs allowance; assets under \$2,000
SSI + State Supplement (other levels)	Room & board in an adult home (varies by level/region)	SSI federal benefit \$994 individual / \$1,491 couple, plus SSP by living arrangement; varies by congregate level
Assisted Living Program (ALP) — Medicaid	Personal care / home care services & case management (NOT room & board)	Medicaid rate paid to the facility; capped at 4,200 statewide; UAS-NY assessment; nursing-home level of care; Local DSS approval; not an entitlement
Private pay	ALR services and the balance of costs	Market rate (varies widely by region); ALR-only facilities are essentially private pay

ALP financial eligibility in 2026 is generally income up to about \$1,836/month for an individual (\$2,489 for a couple) with assets under \$2,000 — confirm current limits and the SSI/SSP congregate-care rates with your Local DSS and SSA. Apply for Medicaid and ALP services through the Local Department of Social Services.

PART 12

What It Costs, Finding Residents & Who to Call

In short: Startup depends heavily on your model, building, and region (downstate is far costlier). The building and the long approval timeline are the two biggest financial realities. Residents come through defined channels, and your master contact list is [here](#).

12.1 What it costs to start — planning categories

- **Business setup:** legal entity, EIN, business banking, the NY LLC publication cost, and the workers' comp and disability insurance New York requires.
- **Building to code:** purchase or lease plus construction/renovation to meet the standards and fire code for your type and capacity — usually the largest, most variable cost.
- **Professional & application costs:** architect and a CPA (for feasibility), legal/consulting help with the Common Application, and local permit fees.
- **Carrying costs during approval:** because approval can take 18 months to two years, budget to carry the property and project through the review period before any revenue.
- **Staffing, insurance & reserves:** recruiting, screening, training (including ALR/dementia/Justice Center where applicable), coverage, and working-capital reserves for the months before SSI/SSP and any ALP payments stabilize.

12.2 Finding and admitting residents

Residents come through defined channels: your Local DSS (which handles Medicaid, ALP approval, and SSI/State-Supplement matters and is aware of people needing placement), hospital discharge planners and social workers, nursing facilities discharging to a lower level of care, physicians, the Long-Term Care Ombudsman and area agencies on aging, and families searching directly (the NYSDOH Adult Care Facility directory lists licensed facilities). Admit only residents whose assessed needs you may lawfully meet within your license — adult care facilities may not admit or retain people who need continuous nursing or medical care — complete the assessment and residency agreement, and start records and the service plan from day one.

10.3 Your first-90-days referral plan

Referrals don't happen by accident — they come from a handful of relationships you build on purpose. Here's a simple plan for your first 90 days:

1. **Make a target list.** Write down the 10–15 hospital discharge planners, skilled home-health and hospice liaisons, senior-community directors, and case managers in your service area — these are the people who send families to agencies.
2. **Show up in person with a one-pager.** A clean one-page sheet: who you are, what you do, your service area, how fast you can start, your insurance/credentials, and your phone number. Leave it; then follow up a week later.
3. **Be absurdly responsive.** Answer every call, say yes to same-day starts whenever you can, and report back to the referrer after the first visit — that single habit earns the next referral.
4. **Ask for feedback and the next referral.** After a good start, ask the referrer what would make you easier to work with, and whether they have another family who needs help. Then do it again next week.

12.3 Who to call & exactly what to ask

Who	Contact	What to verify / ask
DOH — Bureau of Licensure & Certification	health.ny.gov	The Common Application; the pre-application conference; Part I/II; current fees & timeframes
Your Local Department of Social Services	your county DSS	Medicaid, ALP approval, and SSI/State-Supplement for your residents
NY Department of State — Corporations	dos.ny.gov	Forming your LLC (\$200); the publication

		requirement; the biennial statement
IRS · NPPEs	irs.gov · nppes.cms.hhs.gov	Your EIN (free); your Type 2 NPI (for the ALP)
Insurance agent (senior-living program)	trustedchoice.com; carriers in Part 3.5	Liability + abuse + property + workers' comp + NY disability
Long-Term Care Ombudsman	ltombudsman.ny.gov	Posting the Ombudsman contact with your grievance procedure

Common questions, answered

Do I have to take Medicaid?

No. Many successful agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — it's often the smarter way to start, because you earn immediately without the enrollment, contracting, and EVV that Medicaid requires. You can add Medicaid later once you have cash flow and a team.

Can I run this from home at first?

Often yes for a small non-medical agency — the care happens in the client's home, so you may not need a storefront to start. Confirm your local zoning and your license/registration rules (some require a business address), and know you'll want real office space as you grow.

Should my caregivers be employees or independent contractors?

Almost always W-2 employees. Classifying caregivers as 1099 contractors to save on taxes is one of the most common — and most expensive — mistakes in home care; it invites wage-and-hour and tax liability, and most payers and insurers require employees. Treat them as employees and carry workers' comp.

How much cash do I really need to start?

Enough to cover several weeks — ideally a couple of months — of caregiver payroll before your first payer reimbursements arrive, plus your license/insurance/software setup. Under-capitalization is the number-one reason new agencies fail; line up a business line of credit before you scale.

How long until it's profitable?

Most agencies take several months to a year to build a steady client base and referral flow. Starting on private pay shortens the runway because you're paid quickly; Medicaid volume comes later and pays slower. Plan your finances for a ramp, not an overnight full census.

Do I need experience in health care?

It helps, but plenty of successful owners come from business, sales, or family-caregiving backgrounds and hire the clinical expertise they need (for example, a nurse where the state requires one). What matters more is being organized, responsive, good with people, and disciplined about compliance and cash.

What actually makes one agency win over another?

Reliability. Families and referral sources choose — and stay with — the agency that answers the phone, starts care fast, sends the same dependable caregiver, communicates, and never leaves a shift unstaffed. Get that right and referrals compound; get it wrong and no amount of marketing saves you.

What's the difference between an ACF, an ALR, and the ALP?

They're a stack. The ACF (adult home or enriched housing) is the base license for room, board, supervision, and personal care. The ALR is a certification you add for the modern assisted-living model (24-hour monitoring, service plans, daily food service). The ALP is a Medicaid service overlay — capped and competitive. Different layers, different payers — decide which you need before you design the building or the budget.

Can I just apply for the ALP to get Medicaid funding?

No — the ALP is capped at 4,200 residents statewide, is not an entitlement, and new beds are released only through periodic competitive Requests for Applications from DOH. If Medicaid-funded services are central to your plan, you watch for a solicitation and compete for capacity. An ALR without the ALP is essentially private pay.

Does Medicaid pay for room and board?

No. In New York, room and board is paid from the resident's own income — SSI plus the New York State Supplement for low-income residents — and Medicaid (through the ALP) pays only for services. Be precise with families: two different sources pay the two halves of the bill, and the resident keeps a personal needs allowance.

How long does it really take to open?

Plan for 18 months to two years. New construction has to move through architectural/plan review, local permits, and DOH's two-part establishment process, and you can't admit anyone until both Part I and Part II are approved. Take the pre-application conference, assemble complete character-and-competence and feasibility packages, and budget to carry the project through a long, revenue-free approval period.

You can do this

Opening a New York adult care facility or assisted living residence is one of the more involved paths in senior care — a layered license, a long approval, and close scrutiny of your finances and character — but the demand and the funding structure are real. Decide your full license stack before you design the building, take the pre-application conference, assemble complete feasibility and character packages, build to code with DOH and your architect early, and plan your payer mix around SSI/SSP and the capped ALP. Verify each fee and contact as you go, and lean on the people in Part 12.

***Reminder.** Prepared for PolicyReady.org and grounded in SSL Article 7, 18 NYCRR Parts 485–490, PHL Article 46-B, 10 NYCRR Part 1001, 18 NYCRR Part 494, and current NY Medicaid / SSP information as of 2026. This is general information, not legal, tax, or clinical advice. Requirements, fees, rates, and contacts change — verify each with the Department of Health, your Local Department of Social Services, and qualified counsel before you rely on it. Confirm the current Common Application requirements, the SSI/SSP congregate rates, and ALP availability before you rely on them.*